Family-level factors contributing to diet behaviors among Hispanics: opportunities for nutrition intervention

Laboratory for Community Nutrition and Cardiometabolic Health

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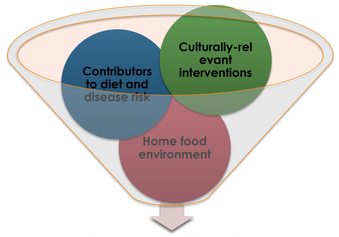
About My Lab

The Community Nutrition and Cardiometabolic Health Laboratory focuses on the role of lifestyle behaviors and dietary quality on cardiometabolic disease prevention among Hispanic individuals and families, with particular interest in the following areas:

1. Identifying factors contributing to diet quality
2. Assessing the role of the home food environment on dietary behaviors and as an intervention target for diet quality improvement
3. Designing, implementing, and evaluating culturally-relevant interventions for cardiometabolic disease risk reduction



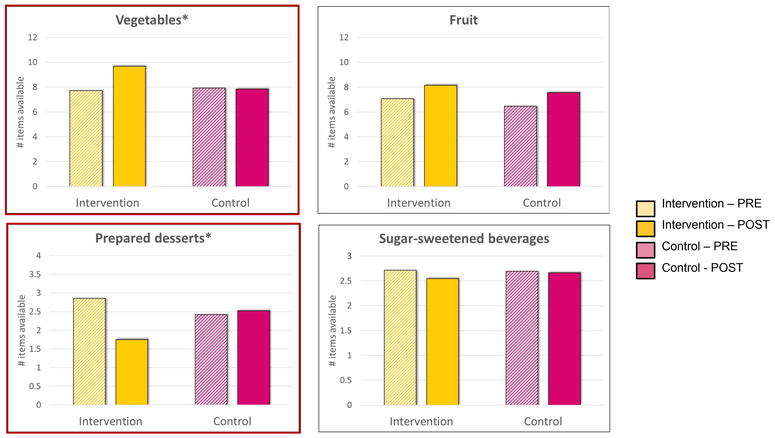
Cardiometabolic health promotion/disease prevention among Hispanics

Why focus on family?

- **Shared and learned behaviors** – Lifestyle behaviors are learned at home and often shared among multiple family members
- **Extended intervention effects** to more than one member of the family
- **Motivation** – Interest in being present and available to other family members
- **Role of parents** – Parents play a central role in promoting healthful behaviors due to these roles:
  - Behavior models
  - Resource providers
  - Ability to shape the home food environment
- **Household context**
  - Rules and feeding practices
  - Family meals
  - Food availability
- **Cultural sensitivity** – Family plays a central role in support structures among Hispanic communities

Home food environment

Results from a randomized controlled trial assessing the efficacy of a community-based diet and physical activity intervention for families with elementary school aged children, *Athletes for Life*, that incorporated content related to improving the home food environment (n=131 households).[1,2]



*p<0.05

No differences in change for, high-sugar breakfast cereals, candy, or sugar-free beverages.

Parenting practices

Associations of self-reported feeding practices (parental monitoring, family eating habits, parental feeding style, parental conversations regarding weight) with adolescent cardiometabolic risk factors (BMI%ile and Z score, systolic and diastolic blood pressure, total cholesterol, and HbA1c; n=61 dyads).[5]

Parents	Mean ± SD or n (%)
Age (years)	40.6 ± 5.6
Female	57 (91%)
Married	36 (58%)
Annual Income	~ $20,000
Household Size	5.3 ± 2.0

Adolescents	Mean ± SD or n (%)
Age (years)	12.4 ± 1.1
Female	27 (44%)
BMI %	83 ± 20
BMIz	1.2 ± 0.9
SBP (mm Hg)	106 ± 11
DBP (mm Hg)	62 ± 7
Total-C (mg/dL)	164 ± 12
HbA1c (%)	5.1 ± 0.4

Use of Media During Meals (TV, electronic devices, etc.)	B	r	p
Weight	7.8	.31	.02
BMIz	.37	.27	.05

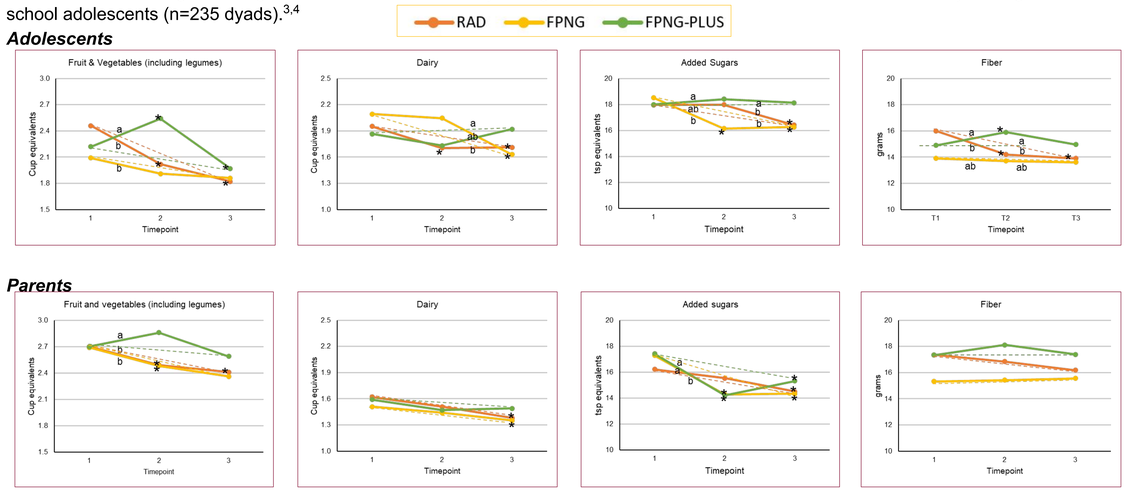
Parental Restrictive Feeding Style	B	r	p
Weight	5.5	.22	.03
BMI%	13.2	.52	.001
DBP	2.5	.27	.04

Parental Energy Index	B	r	p
Weight	6.9	.46	.001
BMI%	6.3	.33	.01

Parent-reported frequency of consuming family meals, frequency of fast food meals, importance of consuming family meals, and inclusion of salad, vegetables, 100% fruit juice, fruit, milk, and sugar-sweetened beverages during family meals were not associated with adolescents' risk factors.

Parenting

Results from a randomized controlled trial assessing the efficacy of *Families Preparing the New Generation Plus*, a 10-week parenting intervention enhancing parenting practices to promote healthy eating and youth substance use prevention simultaneously among middle school adolescents (n=235 dyads).[3,4]



Asterisks (*) indicate significant within-group differences in intake at T2 or T3 relative to T1.

Different superscript letters among T2-T1 and T3-T1 trend lines indicate significant between group changes in intake.

Conclusions

Focusing on family health and the broad household environmental context is a promising approach for promoting healthy eating among Hispanics.

References

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